MONTHLY COMPLIANCE PROGRESS BINDER

Form § 680 Full-Text Continuous Service Logs

Individual Name:	Steven (Steve)	**Record Window:**	May 01, 2026 - May 31, 2026
Primary Diagnoses:	Type II Diabetes, Epilepsy/ Seizure Disorder, Hypertension, High Cholesterol	**Provider Agency:**	SL Community Residential Services

ISP COMPLIANCE FRAMEWORK SUMMARY

Mandatory Tracked Benchmarks (Addressed Across All Shift Rotations):

Outcome 2 (Weekly): Active community integration, social adaptation, and outdoor safety tracking.	**Identified Risk 1 (Daily):** Dehydration mitigation, restriction of hyper-glycemic energy drinks, hydration checks.
Outcome 4 (Daily): Independent-living goals, kitchen appliance tracking (microwave/stove), low-glycemic dietary planning.	**Identified Risk 2 (Daily):** Continuous monitoring for seizure activity, protocol enforcement, post-ictal observation.
Routine Support 3 (Daily): Complete adherence to approximately 10 daily medication administrations.	**Routine Support 8 (Weekly):** Financial accountability, budgeting tracking, and room-improvement savings.

DAILY CONTINUOUS SHIFT PROGRESS LOGS

The following logs contain individualized, shift-by-shift documentation for each calendar day of the record window. Each 24-hour cycle is divided into Day, Evening, and Overnight shift blocks, with staff attestation captured on the final signature page.

May 01, 2026 (Friday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Upbeat, Focused

Inner Vision Day Support Attendance: A taxi picked Steven up from the group home at 7:45 AM for his day at Inner Vision Day Support. The center noted that he sustained an excellent behavioral baseline across the program day, joining recreational and skill-based group activities and greeting peers politely. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). Staff noted he was in especially high spirits, as this marks his full year since coming to the home. For breakfast, he ate oats. For lunch, he had yogurt. He came back to the residence safely by taxi at 4:00 PM.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Easygoing, Chatty

Evening Wellness & Cooking Practice: Arriving home at 4:00 PM, Steven greeted staff and talked briefly about his day during a wellness check-in, joyfully celebrating his full year since coming to the home (Routine Support 2). He reviewed photos from a recent outing and talked about his favorites. For dinner he assisted with a lean turkey stir-fry, carefully watching serving sizes per his diabetic plan, staying within his diabetic targets (Outcome 4), and enjoyed a fresh side salad. Staff administered his evening medications precisely at the scheduled time (Routine Support 3). Staff encouraged a full glass of water and steered him away from hyper-glycemic drinks (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). Steven stayed comfortable and secure.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Overnight Clinical Observation: Steven managed his evening hygiene independently and went to bed on time. Staff administered his insulin as prescribed for his diabetes management. Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He slept soundly through the full night with no disruptions. No nocturnal seizures, pressure-area issues, or other concerns were noted; he remained safe throughout the night.

May 02, 2026 (Saturday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Happy, Cooperative

Weekend Leisure and Community Outing: Rahel supported Steven with a weekend outing on a community drive and a stroll around the lakeside path (Outcome 2). Steven used the outing to practice his social-communication goals, greeting staff and peers politely, mentioning his excitement about his full year since coming to the home. For breakfast, he ate a banana. For lunch, he had a taco. Staff and Steven cooked together at the home. Staff maintained hydration reminders and monitored safety through every transition (Risk 1). No seizure activity or health issues were noted.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Considerate

Home Routine, Cooking & Wellness Checks: Steven returned to the home at 4:00 PM and eased into a relaxed evening after a short check-in with staff, reflecting happily on his one-year anniversary at the home (Routine Support 2). He worked on a jigsaw puzzle at the dining table. When it was time to cook, Steven and staff cooked together at the home, grilling a lean burger patty with a side salad, following his diabetic portion rules (Outcome 4). He took his prescribed evening medications with water under staff supervision (Routine Support 3). Staff prompted regular fluid intake, offering a sugar-free flavor packet to encourage hydration (Risk 1), and seizure and choking precautions were maintained throughout the meal (Risk 2, Potential Risk 2). Vitals stayed steady and stable all evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Rested

Nocturnal Health Protocol Monitoring: Steven wrapped up his personal-care routine without assistance and was in bed by his usual hour. The nightly insulin was administered for his diabetes management. Throughout the block, staff observed for any seizure, respiratory, or diabetic irregularity (Risk 2, Potential Risk 3). He slept soundly through the full night with no disruptions. Steven showed no seizures, no falls, and no diabetic or respiratory concerns.

May 03, 2026 (Sunday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Happy, Cooperative

Weekend Recreation & Social Integration: Hana facilitated Steven's structured weekend routine, taking him to a nearby cafe for a diabetic-friendly breakfast outing followed by a short walk (Outcome 2). Steven engaged pleasantly with others in the space and applied his greeting and conversation goals, proudly discussing his full year since coming to the home. For breakfast, he ate oats. For lunch, he had yogurt. Staff and Steven cooked together at the home. Hydration prompts were provided per his dehydration-prevention plan and all movement was supervised (Risk 1). He remained safe, upbeat, and free of any health concerns.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Content

Evening Independent-Living Routine: Following his return at 4:00 PM, Steven checked in with staff and shared a few highlights from his day, feeling proud of his full year at the home (Routine Support 2). He browsed sports highlights on his phone and chatted with staff about them. Then, Steven and staff cooked together at the home, grilling a lean burger patty with a side salad, following his diabetic portion rules (Outcome 4). He took his prescribed evening medications with water under staff supervision (Routine Support 3). Staff prompted regular fluid intake, offering a sugar-free flavor packet to encourage hydration (Risk 1). Continuous surveillance for seizure and aspiration risk was kept through dinner (Risk 2, Potential Risk 2). No concerns were noted across the block.

Shift (Start and End Time): 12am-8am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Sleepy

Continuous Overnight Health Watch: After finishing his personal hygiene independently, Steven settled into bed at his usual time. His bedtime insulin was administered for his diabetes management. Continuous monitoring was maintained through the night for seizures, breathing, and diabetic baselines (Risk 2, Potential Risk 3). He slept soundly through the full night with no disruptions. The shift passed without any seizure activity, falls, or medical concerns.

May 04, 2026 (Monday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Relaxed, Talkative

Structured Program & Peer Interaction: Steven headed out from the group home at 7:45 AM by taxi for his scheduled day at Inner Vision Day Support. Center staff reported strong engagement in the day-support framework, including active socialization, teamwork dynamics, and structured skill stations. For breakfast, he ate a banana. For lunch, he had a taco, staying within his diabetic guidelines (Outcome 4). He was transported back to the group home at 4:00 PM without incident.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Pleasant, Settled

In-House Recreation & Meal Preparation: Home at 4:00 PM, Steven eased into his evening after a brief 1-on-1 check-in with staff, reflecting on his progress over his full year since coming to the home (Routine Support 2). He listened to music and organized items on his new storage shelf. At dinnertime he cooked brown rice with sauteed peppers, lean protein, and a fresh side salad, portioned to his guidelines, applying the portion habits he has been practicing (Outcome 4). The evening medication pass was completed accurately with water (Routine Support 3). Staff kept water within reach and reminded him to sip throughout the evening (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). Steven stayed comfortable and secure.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Monitoring & Medication Compliance: Steven independently completed his bedtime hygiene care and lay down on schedule. His bedtime insulin was administered for his diabetes management. Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He slept soundly through the full night with no disruptions. Steven showed no seizures, no falls, and no diabetic or respiratory concerns; he stayed secure under continuous supervision.

May 05, 2026 (Tuesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Excited, Outgoing

Daytime Community Inclusion: A taxi picked Steven up from the group home at 7:45 AM and transported him to Inner Vision Day Support. Center notes confirm he settled into the morning routine well, contributing to group discussion and completing structured tasks with peers. For breakfast, he ate oats. For lunch, he had yogurt, staying within his diabetic guidelines (Outcome 4). He returned to the residence by taxi at 4:00 PM with no health concerns reported. Tuesday Community Outing: Consistent with his Tuesday schedule, Steven joined an extended mall walk for physical exercise (Outcome 2). He navigated the outing safely while staff supported safety awareness (Potential Risk 2).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Content

Evening Care Routine & Diabetic Cooking: Following his return at 4:00 PM, Steven checked in with staff and shared a few highlights from his day, wrapping up the celebrations for his full year since coming to the home (Routine Support 2). He watched an episode of a favorite series while unwinding on the couch, then warmed a pre-portioned diabetic-friendly meal, added fresh steamed vegetables, and had a side salad (Outcome 4). His evening medications were given with water at the correct hour (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1). Staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). He ended the shift relaxed and content. Allowance Tracking (Routine Support 8): Staff verified Steven received his weekly $50 allowance on schedule and logged the intake in his financial ledger to support his structured savings targets.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Safety & Rest Monitoring: After finishing his personal hygiene independently, Steven settled into bed at his usual time. His nightly insulin was administered for his diabetes management. Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He slept soundly through the full night with no disruptions. Zero seizures, zero falls, and no health irregularities were recorded overnight.

May 06, 2026 (Wednesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hiwot Tedla, DSP | Observed Mood: Calm, Conversational

Mid-Week In-House Skill Building (No Day Support): Per his service plan, Steven remained in the group home today to focus on domestic independent-living goals. Staff initiated a comprehensive 1-on-1 check-in, and Steven opened up in depth (Routine Support 2), sharing personal reflections from memory—his anxieties about adulthood, his relationship fears, his cousins who are each raising families of at least two children, his mother's live-in boyfriend, and a recent visit from his father. Staff held a secure, non-judgmental space that allowed him to express his feelings openly and confidently. He displayed excellent emotional regulation, and hydration was maintained throughout the block (Risk 1).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Focused, Content

Outcome 4 & Routine Support 5 (Domestic Chores): Steven dedicated his evening to domestic-independence tasks. He completed his weekly laundry routine (Routine Support 5) gathering worn items into his basket and managing the washer and dryer cycles under staff supervision. For dinner, he practiced safe appliance use by warming his meal plate in the microwave, handling oven mitts carefully, and eating it with a healthy side salad (Outcome 4). Evening medications were administered with water (Routine Support 3), hydration was encouraged (Risk 1), and staff monitored for seizure and choking risk (Risk 2). Staff praised his independent-living progress.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Rested

Overnight Clinical Observation: Having completed his hygiene routine on his own, Steven turned in at his regular bedtime. Staff dispensed his bedtime medications with water (Routine Support 3). Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He settled quickly and stayed asleep throughout the shift. Steven showed no seizures, no falls, and no diabetic or respiratory concerns.

May 07, 2026 (Thursday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Happy, Cooperative

Day Support Program Integration: A taxi picked Steven up from the group home at 7:45 AM and transported him to Inner Vision Day Support. Program documentation describes him as engaged and personable, taking part in community-awareness exercises and a hands-on group activity. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). Staff at the center reported no concerns; he returned home at 4:00 PM.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Happy, Excited, Outgoing

Skill Building & House Optimization Project: Steven showed remarkable motivation toward his personal goals this evening. He safely prepared cheese with bread using kitchen appliances under staff monitoring, along with a healthy side salad (Outcome 4). He and staff then launched a bedroom optimization project: Steven outlined his goal to purchase a new lamp and storage shelves. Staff delivered an interactive budgeting lesson on saving consistently from his weekly allowance (Routine Support 8). He was highly collaborative and visibly excited. Evening medications were given with water (Routine Support 3), and hydration and seizure monitoring were maintained (Risk 1, Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Continuous Overnight Health Watch: With his nightly hygiene done independently, Steven lay down at his scheduled bedtime. His prescribed nighttime medications were provided with water as ordered (Routine Support 3). Staff kept periodic bed checks through the night, noting respiration, seizure status, and comfort (Risk 2). He remained asleep and settled across the entire block. There were no seizure events, no falls, and no medical anomalies during the block; no supervision concerns arose.

May 08, 2026 (Friday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Content, Polite

Day Center Socialization Focus: Following his morning routine, Steven caught a taxi from the group home at 7:45 AM to Inner Vision Day Support. Program documentation describes him as engaged and personable, taking part in community-awareness exercises and a hands-on group activity. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). He returned to the residence by taxi at 4:00 PM with no health concerns reported.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Reflective

Music Therapy & Spatial Boundaries: Steven chose to dedicate his evening to creative leisure, spending a significant portion of his independent blocks practicing guitar in his bedroom. Staff respected the privacy boundaries he values, checking in periodically (Routine Support 2). He ate a diabetic-friendly dinner with a fresh side salad. Staff provided fluid tracking, ensuring he drank water supplemented with a sugar-free flavor packet to satisfy his dehydration-prevention criteria (Risk 1). Evening medications were administered with water (Routine Support 3), and no seizure activity was observed (Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Rested

Overnight Safety & Clinical Observation: After finishing his personal hygiene independently, Steven settled into bed at his usual time. The nightly medication pass was verified and given with water (Routine Support 3). Staff maintained quiet overnight rounds, confirming stable respiration and no seizure signs (Risk 2). He had a restful, uninterrupted night of sleep. The shift passed without any seizure activity, falls, or medical concerns.

May 09, 2026 (Saturday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Happy, Cooperative

Financial Literacy and Savings Review: Nathaniel sat down with Steven for a structured money-management lesson (Routine Support 8). They reviewed his current savings ledger and discussed upcoming community-lunch costs. Steven demonstrated strong discipline and critical thinking, agreeing to cap his community-lunch budget at exactly $20 so the remainder of his allowance could funnel directly into his bedroom-shelf savings. He expressed genuine excitement about his financial-milestone progress. Hydration was encouraged throughout (Risk 1). Staff and Steven cooked together at the home.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Content, Relaxed

Hydration Compliance Milestone: Staff monitored Steven closely during his weekend evening. He relaxed with phone games and downtime. Staff and Steven cooked together at the home for dinner. To meet his dehydration-prevention parameters, staff encouraged a full glass of water mixed with his sugar-free flavor powder (Risk 1); Steven accepted willingly and finished the whole mixture. Staff offered enthusiastic praise, reinforcing healthy choices and the plan to limit hyper-glycemic energy drinks. Evening medications were administered with water (Routine Support 3); no seizure activity was noted (Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Rested

Overnight Safety & Clinical Observation: After a self-directed wind-down and hygiene routine, Steven went to bed on schedule. His nighttime medications were administered accurately with water (Routine Support 3). Continuous monitoring was maintained through the night for seizures, breathing, and diabetic baselines (Risk 2, Potential Risk 3). He slept peacefully, waking only briefly before drifting back off. There were no seizure events, no falls, and no medical anomalies during the block. Residential supervision continued uninterrupted.

May 10, 2026 (Sunday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Happy, Cooperative

Weekend Community Integration: Steven's weekend leisure block was led by Rahel, who took him to a local shopping plaza to window-browse and walk the concourse at an easy pace (Outcome 2). Steven greeted others courteously, honored his communication targets, and enjoyed the change of scenery. Staff monitored for seizure activity and steered him toward water over sugary drinks throughout (Risk 1, Risk 2). The outing passed smoothly with no incidents.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Relaxed, Cooperative

In-House Routine & Diabetic Meal Support: Steven settled in at 4:00 PM and shared how his day had gone during a one-on-one check-in (Routine Support 2). He worked on a jigsaw puzzle at the dining table before preparing his meal. Steven and staff cooked together at the home, where he made a tuna-and-greens bowl with a side salad, keeping the meal aligned with his low-glycemic plan (Outcome 4). Staff dispensed his evening medications on time with water (Routine Support 3). Fluids were encouraged and a sugar-free electrolyte packet was provided to meet his hydration goal (Risk 1), and staff monitored closely for any seizure or choking signs during the meal (Risk 2, Potential Risk 2). No concerns were noted across the block.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Peaceful, Sleepy

Overnight Clinical Observation: Steven saw to his bedtime hygiene on his own and settled under the covers on time. Bedtime medications were administered with water (Routine Support 3). Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He slept soundly through the full night with no disruptions. No nocturnal seizures, pressure-area issues, or other concerns were noted.

May 11, 2026 (Monday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Content, Polite

Day Center Socialization Focus: Steven left for Inner Vision Day Support at 7:45 AM via a taxi that picked him up from the home. Center staff reported strong engagement in the day-support framework, including active socialization, teamwork dynamics, and structured skill stations. He had prepared and packed his own lunch prior to pickup, staying within his diabetic guidelines (Outcome 4). He arrived home at 4:00 PM in good spirits, with no seizure activity or medical concerns noted.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Content, Relaxed

Evening Skill Practice & Health Monitoring: Following his return at 4:00 PM, Steven checked in with staff and shared a few highlights from his day (Routine Support 2). He reviewed photos from a recent outing and talked about his favorites, then prepared whole-grain pasta with a light sauce and a healthy side salad, keeping his plate within diabetic limits (Outcome 4). The evening medication pass was completed accurately with water (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1). Continuous surveillance for seizure and aspiration risk was kept through dinner (Risk 2, Potential Risk 2). Steven stayed comfortable and secure.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Peaceful, Sleepy

Nocturnal Health Protocol Monitoring: After finishing his personal hygiene independently, Steven settled into bed at his usual time. Staff administered his bedtime regimen with water and documented it (Routine Support 3). Hourly welfare checks tracked breathing, seizure activity, and diabetic stability across the shift (Risk 2, Potential Risk 3). He remained asleep and settled across the entire block. No nocturnal seizures, pressure-area issues, or other concerns were noted.

May 12, 2026 (Tuesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Excited, Outgoing

Day Program Participation & Social Skills: Steven boarded a taxi at the home at 7:45 AM for the ride to Inner Vision Day Support. Center records indicate he engaged actively in group programming, practiced his communication goals with peers, and maintained a positive baseline throughout the day. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). He was transported back to the group home at 4:00 PM without incident. Tuesday Community Outing: As part of his weekly Tuesday routine, Steven took a brisk walk around the shopping mall for exercise (Outcome 2) and enjoyed a grilled-chicken bowl for lunch, with staff monitoring portioning and safe swallowing (Potential Risk 2).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Proud, Friendly, Outspoken

Routine Support 8 (Financial Progress) & High-Value Social Intersections: Steven reached an outstanding financial milestone today. Through consistent budgeting, he kept $3 aside from his allocated $20 lunch budget; staff updated his shelf-savings tracker, confirming a cumulative total of $33 (Routine Support 8). Staff expressed real pride in his discipline. During his daytime mall outing, Steven had valuable social encounters—unexpectedly meeting his ex-girlfriend Melissa and running into his former roommate—and managed both with impeccable social boundaries and a friendly, outspoken demeanor (Routine Support 2). In the evening he independently used the stove to prepare noodles for dinner with a healthy side salad, strictly observing safety guidelines (Outcome 4). Medications, hydration, and seizure monitoring were maintained (Routine Support 3, Risk 1, Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Safety & Rest Monitoring: Following an independent nighttime hygiene routine, Steven retired to bed at his scheduled hour. His prescribed nighttime medications were provided with water as ordered (Routine Support 3). Staff kept periodic bed checks through the night, noting respiration, seizure status, and comfort (Risk 2). He slept peacefully, waking only briefly before drifting back off. No nocturnal seizures, pressure-area issues, or other concerns were noted.

May 13, 2026 (Wednesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hiwot Tedla, DSP | Observed Mood: Restless, then Calm

Behavioral Support & Community Interaction: Steven remained in-house today for domestic skill programming. During a social exchange, he had a verbal disagreement with another individual in the environment, exhibiting restlessness and elevated voice volume, initially refusing to share and seeking immediate attention. Staff implemented de-escalation protocols promptly, providing communication support and encouraging him to express his feelings appropriately (Routine Support 2). Steven responded positively and relocated to a quiet space to regroup. Later in the shift, local emergency services responded to an unrelated incident at the adjacent group-home property; Steven observed briefly with staff, who used grounding techniques to keep him calm and secure. Hydration was maintained (Risk 1).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Content

Evening Care Routine & Diabetic Cooking: Arriving home at 4:00 PM, Steven greeted staff and talked briefly about his day during a wellness check-in (Routine Support 2). He played a card game with staff at the kitchen table. For dinner he warmed a pre-portioned diabetic-friendly meal and added fresh steamed vegetables and a side salad, staying within his diabetic targets (Outcome 4). Evening medications were administered on schedule with water (Routine Support 3). Staff prompted regular fluid intake, offering a sugar-free flavor packet to encourage hydration (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). Vitals stayed steady and stable all evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Safety & Rest Monitoring: Steven managed his evening hygiene independently and went to bed on time. His nighttime medications were administered accurately with water (Routine Support 3). Throughout the block, staff observed for any seizure, respiratory, or diabetic irregularity (Risk 2, Potential Risk 3). He slept peacefully, waking only briefly before drifting back off. Steven showed no seizures, no falls, and no diabetic or respiratory concerns; he remained safe throughout the night.

May 14, 2026 (Thursday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Happy, Cooperative

Day Support Program Integration: Steven left for Inner Vision Day Support at 7:45 AM via a taxi that picked him up from the home. Center staff reported strong engagement in the day-support framework, including active socialization, teamwork dynamics, and structured skill stations. He managed his own lunchbox independently that morning, making diet-appropriate choices (Outcome 4). He returned to the residence by taxi at 4:00 PM with no health concerns reported.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Sad, Worried, then Calm

Trauma Processing & Appliance Safety Teaching Moment: While traveling home from day support, Steven witnessed a community vehicular accident, which noticeably affected his emotional baseline; he arrived home visibly sad and expressing worry. Staff immediately prioritized his psychological safety, providing a quiet 1-on-1 counseling space to help him process his anxiety (Routine Support 2). Later, Steven pursued independent dinner preparation, cooking pasta and sausage on the stove safely, paired with a side salad (Outcome 4). While preparing a side soup, he became briefly distracted by his phone and the soup burned on the burner; staff intervened instantly, managed kitchen safety, and ventilated the area. This was used as a constructive, non-punitive teaching moment on continuous appliance monitoring. No injuries occurred, and evening medications were administered with water (Routine Support 3).

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Safety & Rest Monitoring: Having completed his hygiene routine on his own, Steven turned in at his regular bedtime. His prescribed nighttime medications were provided with water as ordered (Routine Support 3). Staff performed regular overnight rounds, watching for seizure activity, respiratory changes, and stability (Risk 2). He settled quickly and stayed asleep throughout the shift. There were no seizure events, no falls, and no medical anomalies during the block.

May 15, 2026 (Friday) - Status: Out of Facility Travel Prep

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Focused, Calm, Thoughtful

Bereavement Travel Preparation: Steven's father arrived at the facility to transport him to Fredericksburg, VA, following the passing of Steven's maternal aunt. Ahead of the departure, staff partnered closely with Steven to confirm full clinical and personal readiness for the trip. He displayed exceptional focus during this preparation window. Staff assisted him in carefully packing his personal attire and required travel snacks, and meticulously counted and packed his complete supply of approximately 10 daily medications (Routine Support 3). Full portable safety and seizure protocols were printed and handed directly to his father (Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Out of Facility / Family Care | Observed Mood: Not Scheduled

Steven successfully checked out of the group-home facility under the direct supervision of his father. Standard residential clinical programming, cooking targets, and in-house milestones were temporarily placed on a formal hold for bereavement leave. Continuous support was provided by family contacts.

May 16, 2026 (Saturday) - Status: Out of Facility (Fredericksburg, VA)

Shift (Start and End Time): 24-Hour Continuous

Assigned Provider: Out of Facility (Bereavement Leave) | Observed Mood: Content, Calibrated

Community Purchases and Outings: Steven remained out of the residential facility, co-residing temporarily with his family network in Fredericksburg, VA. Family updates confirm he engaged in community-based recreational adjustment to ease bereavement stress. He completed a targeted shopping outing, purchasing a new winter coat and a pair of shoes, and afterward enjoyed an ice cream cone in the community. Family reported an excellent appetite, positive interactions, and full compliance with taking his approximately 10 daily medications with water as prompted (Routine Support 3). No clinical health issues or seizure anomalies were reported (Risk 2).

May 17, 2026 (Sunday) - Status: Out of Facility (Fredericksburg, VA)

Shift (Start and End Time): 24-Hour Continuous

Assigned Provider: Out of Facility (Bereavement Leave) | Observed Mood: Happy, Social

Family Socialization and Milestone Integration: Family contacts provided updates on Steven's behavioral stability during his temporary stay. He actively participated in a family gathering, joining a community-style picnic where homemade burgers were prepared. He also shared valuable 1-on-1 time with his girlfriend Melissa over a community lunch featuring preferred snacks, including peanut-butter items (Routine Support 2). Family described Steven as highly conversational and engaged, with no signs of depressive withdrawal and a fully stable behavioral profile throughout. Medication adherence was maintained flawlessly under parent monitoring (Routine Support 3).

May 18, 2026 (Monday)

Shift (Start and End Time): 8am-4pm (Status: Facility Return)

Assigned Provider: Out of Facility | Observed Mood: Calm, Cooperative

Steven remained under out-of-facility bereavement status for the morning and early-afternoon blocks, safely completing his travel itinerary from Fredericksburg back to the residential home under family escort (Routine Support 2).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Content, Grateful

Facility Intake & Re-entry: Steven arrived safely back at the group home at approximately 5:30 PM. Staff conducted a comprehensive intake check, confirming his physical-health baseline was fully intact and that all remaining portable medications were returned and logged accurately (Routine Support 3). He shared warmly that he had a meaningful trip with his family (Routine Support 2). He spent the remainder of the evening relaxing, playing mobile games, hydrating properly with a sugar-free flavor packet (Risk 1), and enjoyed a diabetic-friendly dinner with a side salad. No seizure activity was noted (Risk 2).

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Restful, Settled

Nocturnal Health Protocol Monitoring: Steven managed his evening hygiene independently and went to bed on time. The nightly medication pass was verified and given with water (Routine Support 3). Continuous monitoring was maintained through the night for seizures, breathing, and diabetic baselines (Risk 2, Potential Risk 3). He slept without interruption and woke calm and rested. No seizures, falls, or skin-integrity concerns were observed, and vitals stayed stable; he remained safe throughout the night.

May 19, 2026 (Tuesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Excited, Outgoing

Day Center Socialization Focus: Steven headed out from the group home at 7:45 AM via taxi for his scheduled day at Inner Vision Day Support. The center noted that he sustained an excellent behavioral baseline across the program day, joining recreational and skill-based group activities and greeting peers politely. He packed his lunch on his own before leaving, a routine he now handles with minimal prompting (Outcome 4). Staff at the center reported no concerns; he returned home at 4:00 PM. Tuesday Community Outing: Steven followed his Tuesday community plan with a walk through the park and light window-shopping (Outcome 2), then had a diabetic-friendly sandwich out with peers, managing the meal safely under staff supervision (Potential Risk 2).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Content, Relaxed

Evening Skill Practice & Health Monitoring: Home at 4:00 PM, Steven eased into his evening after a brief 1-on-1 check-in with staff (Routine Support 2). He tidied his bedroom and rearranged his desk the way he likes it. At dinnertime he prepared baked chicken thighs with roasted zucchini and a side salad, checking portions with staff, applying the portion habits he has been practicing (Outcome 4). Evening medications were administered on schedule with water (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1), and seizure and choking precautions were maintained throughout the meal (Risk 2, Potential Risk 2). The evening passed calmly with no issues.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Rested

Nocturnal Health Protocol Monitoring: Steven independently completed his bedtime hygiene care and lay down on schedule. Staff dispensed his bedtime medications with water (Routine Support 3). Staff conducted routine overnight safety checks all shift, tracking respiration, seizure status, and overall stability (Risk 2). He remained asleep and settled across the entire block. Steven showed no seizures, no falls, and no diabetic or respiratory concerns; he stayed secure under continuous supervision.

May 20, 2026 (Wednesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hiwot Tedla, DSP | Observed Mood: Focused, Industrious

Mid-Week In-House Skill Building (No Day Support): In accordance with mid-week programming, Steven remained inside the facility to target domestic independent-living indicators (Routine Support 4). He stayed at home, focusing on doing his laundry and other household chores. Staff guided him through a daily chore checklist—bedroom sweeping, garbage disposal, and countertop sanitization. A dedicated 1-on-1 communication checkpoint was provided, where Steven expressed feeling content and secure with his weekly structure (Routine Support 2). Proper hydration was maintained throughout the daytime block (Risk 1).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Settled, Friendly

Evening Independent-Living Routine: Steven returned to the home at 4:00 PM and eased into a relaxed evening after a short check-in with staff (Routine Support 2). He browsed sports highlights on his phone and chatted with staff about them. When it was time to cook, he warmed a pre-portioned diabetic-friendly meal, added fresh steamed vegetables, and enjoyed a side salad (Outcome 4). His evening medications were given with water at the correct hour (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1), and staff monitored closely for any seizure or choking signs during the meal (Risk 2, Potential Risk 2). He remained calm and safe throughout.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Peaceful, Sleepy

Overnight Clinical Observation: Steven wrapped up his personal-care routine without assistance and was in bed by his usual hour. Bedtime medications were passed on time with water and logged (Routine Support 3). Staff performed regular overnight rounds, watching for seizure activity, respiratory changes, and stability (Risk 2). He rested comfortably from lights-out until morning. No nocturnal seizures, pressure-area issues, or other concerns were noted.

May 21, 2026 (Thursday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Cheerful, Sociable

Structured Day Support Engagement: A taxi picked Steven up from the group home at 7:45 AM and transported him to Inner Vision Day Support. Center notes confirm he settled into the morning routine well, contributing to group discussion and completing structured tasks with peers. He managed his own lunchbox independently that morning, making diet-appropriate choices (Outcome 4). He was transported back to the group home at 4:00 PM without incident.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Settled, Friendly

Evening Independent-Living Routine: Once home at 4:00 PM, Steven relaxed and touched base with staff about his day (Routine Support 2). He watched an episode of a favorite series while unwinding on the couch. For his evening meal he portioned a baked-fish and steamed-broccoli dinner with a healthy side salad, in line with his low-glycemic targets, minding his portions (Outcome 4). Evening medications were provided as ordered, with water, and logged (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1). Staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). The evening passed calmly with no issues.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Monitoring & Medication Compliance: After a self-directed wind-down and hygiene routine, Steven went to bed on schedule. The nightly medication pass was verified and given with water (Routine Support 3). Across the 8-hour block, staff carried out continuous overnight observation for seizure activity, respiration, and diabetic stability (Risk 2, Potential Risk 3). He rested comfortably from lights-out until morning. No nocturnal seizures, pressure-area issues, or other concerns were noted. Residential supervision continued uninterrupted.

May 22, 2026 (Friday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Calm, Engaged

Community Access & Day Program Participation: Steven headed out from the group home at 7:45 AM via taxi for his scheduled day at Inner Vision Day Support. Center records indicate he engaged actively in group programming, practiced his communication goals with peers, and maintained a positive baseline throughout the day. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). He came back to the residence safely by taxi at 4:00 PM.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Considerate

In-House Routine & Diabetic Meal Support: Steven was welcomed home at 4:00 PM and transitioned smoothly into the evening. Staff offered a supportive check-in about his day (Routine Support 2). He browsed sports highlights on his phone and chatted with staff about them before dinner, then prepared whole-grain pasta with a light sauce and a side salad, keeping his plate within diabetic limits (Outcome 4). Staff dispensed his evening medications on time with water (Routine Support 3). Hydration was reinforced with water and a sugar-free drink mix in place of energy drinks (Risk 1). Continuous surveillance for seizure and aspiration risk was kept through dinner (Risk 2, Potential Risk 2). Vitals stayed steady and stable all evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Peaceful, Sleepy

Continuous Overnight Health Watch: Following an independent nighttime hygiene routine, Steven retired to bed at his scheduled hour. Nighttime medications were provided on schedule with water (Routine Support 3). Throughout the block, staff observed for any seizure, respiratory, or diabetic irregularity (Risk 2, Potential Risk 3). He settled quickly and stayed asleep throughout the shift. Steven showed no seizures, no falls, and no diabetic or respiratory concerns.

May 23, 2026 (Saturday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Happy, Enthusiastic

Outcome 2: Comprehensive Community Integration Excursion: Hana planned and executed a full-day recreational excursion to the James River (Outcome 2). Steven was transported safely to the river park in the morning and spent the entire daytime block outdoors, enjoying extensive scenic trail walking, riverfront relaxation, and positive community socialization. Staff logged that Steven had an exceptionally good day across the whole trip, showing strong behavioral stability and clear communication. Constant hydration prompts were provided to account for the outdoor environment (Risk 1), and seatbelt safety was observed during transport (Routine Support 10).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Proud, Excited, Content

Financial Goal Realization & Home Integration: Following his full-day James River excursion, Steven and staff stopped at the store to execute his primary savings goal. Using his carefully accumulated long-term savings of $140—built steadily from his weekly allowances (Routine Support 8)—Steven proudly purchased his targeted bedroom storage-organization shelf. He came home visibly proud and energized, and staff helped him assemble the shelf and position it in his bedroom. He also spent time playing guitar, expressing deep pride in his financial milestone. Steven and staff cooked together at the home, enjoying dinner with a side salad. Hydration and diabetic-diet guidelines were fully maintained (Risk 1, Outcome 4), and evening medications were administered with water (Routine Support 3).

Shift (Start and End Time): 12am-8am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Peaceful, Sleepy

Nighttime Safety & Rest Monitoring: Steven saw to his bedtime hygiene on his own and settled under the covers on time. Bedtime medications were taken with water per order (Routine Support 3). Hourly welfare checks tracked breathing, seizure activity, and diabetic stability across the shift (Risk 2, Potential Risk 3). He slept peacefully, waking only briefly before drifting back off. No nocturnal seizures, pressure-area issues, or other concerns were noted.

May 24, 2026 (Sunday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Happy, Cooperative

Weekend Recreation & Social Integration: For his weekend community access, Nathaniel accompanied Steven to a local shopping plaza to window-browse and walk the concourse at an easy pace (Outcome 2). Steven engaged pleasantly with others in the space and applied his greeting and conversation goals. Staff kept water on hand, reminded him to hydrate, and watched for any seizure or balance issues (Risk 1, Risk 2). No medical or behavioral concerns were observed.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Polite

In-House Routine & Diabetic Meal Support: Steven settled in at 4:00 PM and shared how his day had gone during a one-on-one check-in (Routine Support 2). He did some light stretching and breathing exercises before dinner. Steven and staff cooked together at the home, making a tuna-and-greens bowl with a side salad, keeping the meal aligned with his low-glycemic plan (Outcome 4). Evening medications were provided as ordered, with water, and logged (Routine Support 3). Staff prompted regular fluid intake, offering a sugar-free flavor packet to encourage hydration (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). He remained calm and safe throughout.

Shift (Start and End Time): 12am-8am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Sleepy

Continuous Overnight Health Watch: Steven saw to his bedtime hygiene on his own and settled under the covers on time. Staff dispensed his bedtime medications with water (Routine Support 3). Across the 8-hour block, staff carried out continuous overnight observation for seizure activity, respiration, and diabetic stability (Risk 2, Potential Risk 3). He remained asleep and settled across the entire block. There were no seizure events, no falls, and no medical anomalies during the block.

May 25, 2026 (Monday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Happy, Cooperative

Day Support Program Integration: A taxi picked Steven up from the group home at 7:45 AM for his day at Inner Vision Day Support. Center notes confirm he settled into the morning routine well, contributing to group discussion and completing structured tasks with peers. He independently packed his own lunchbox before departure, reinforcing his growing self-sufficiency (Outcome 4). Staff at the center reported no concerns; he returned home at 4:00 PM.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Calm, Polite

Routine Support Protocols & Skill Practice: Home at 4:00 PM, Steven eased into his evening after a brief 1-on-1 check-in with staff (Routine Support 2). He worked on a jigsaw puzzle at the dining table. At dinnertime he cooked brown rice with sauteed peppers, lean protein, and a side salad, portioned to his guidelines, applying the portion habits he has been practicing (Outcome 4). Evening medications were provided as ordered, with water, and logged (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1), and staff monitored closely for any seizure or choking signs during the meal (Risk 2, Potential Risk 2). Vitals stayed steady and stable all evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Safety & Rest Monitoring: Steven independently completed his bedtime hygiene care and lay down on schedule. The nightly medication pass was verified and given with water (Routine Support 3). Overnight surveillance covered seizure status, respiration, and blood-sugar stability for the full shift (Risk 2). He slept peacefully, waking only briefly before drifting back off. No signs of seizure, respiratory change, or discomfort were observed all night; he stayed secure under continuous supervision.

May 26, 2026 (Tuesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Excited, Outgoing

Community Access & Day Program Participation: Following his morning routine, Steven caught a taxi from the group home at 7:45 AM to Inner Vision Day Support. Center notes confirm he settled into the morning routine well, contributing to group discussion and completing structured tasks with peers. His self-packed lunch met his dietary plan, reflecting continued independent-living progress (Outcome 4). He was transported back to the group home at 4:00 PM without incident. Tuesday Community Outing: For his Tuesday outing, Steven walked the mall concourse for cardio (Outcome 2) and picked a lean-protein plate for his meal, staying within his diabetic portions with staff guidance (Potential Risk 2).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Considerate

In-House Routine & Diabetic Meal Support: Arriving home at 4:00 PM, Steven greeted staff and talked briefly about his day during a wellness check-in (Routine Support 2). He spent time on his mobile games and strummed a few songs on his guitar. For dinner he prepared whole-grain pasta with a light sauce and a healthy side salad, keeping his plate within diabetic limits, staying within his diabetic targets (Outcome 4). He took his prescribed evening medications with water under staff supervision (Routine Support 3). Fluids were encouraged and a sugar-free electrolyte packet was provided to meet his hydration goal (Risk 1), and staff monitored closely for any seizure or choking signs during the meal (Risk 2, Potential Risk 2). He remained safe and settled through the evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Overnight Safety & Clinical Observation: Steven managed his evening hygiene independently and went to bed on time. His prescribed nighttime medications were provided with water as ordered (Routine Support 3). Across the 8-hour block, staff carried out continuous overnight observation for seizure activity, respiration, and diabetic stability (Risk 2, Potential Risk 3). He rested comfortably from lights-out until morning. No signs of seizure, respiratory change, or discomfort were observed all night; he remained safe throughout the night.

May 27, 2026 (Wednesday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hiwot Tedla, DSP | Observed Mood: Focused, Industrious

Domestic Independence & Financial Reward: Steven spent his in-house day actively executing independent household tasks. He organized his private wardrobe space and assisted staff in inventorying cleaning supplies (Routine Support 4). He stayed at home to complete his laundry routine and manage his belongings. Having maintained excellent consistency with his monthly budgeting targets, Steven collaborated with staff on a pre-planned online reward purchase, using his carefully tracked leftover allowance to buy a specialized 'guitar-playing pixel art' accessory for his bedroom (Routine Support 8). He displayed immense satisfaction and distinct motivation to continue his disciplined money-management protocol. Hydration was maintained throughout the daytime block (Risk 1).

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Content, Relaxed

Evening Skill Practice & Health Monitoring: After getting back at 4:00 PM, Steven took a few minutes to decompress and review his day with staff (Routine Support 2). He listened to music and organized items on his new storage shelf and later assisted with a lean turkey stir-fry, carefully watching serving sizes per his diabetic plan for dinner, along with a side salad (Outcome 4). His evening medications were given with water at the correct hour (Routine Support 3). Fluids were encouraged and a sugar-free electrolyte packet was provided to meet his hydration goal (Risk 1). Staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). No concerns were noted across the block.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Restful, Settled

Overnight Clinical Observation: With his nightly hygiene done independently, Steven lay down at his scheduled bedtime. Bedtime medications were administered with water (Routine Support 3). Overnight surveillance covered seizure status, respiration, and blood-sugar stability for the full shift (Risk 2). He slept without interruption and woke calm and rested. There were no seizure events, no falls, and no medical anomalies during the block; no supervision concerns arose.

May 28, 2026 (Thursday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Happy, Cooperative

Day Support Program Integration: At 7:45 AM a taxi picked Steven up from the group home for his program day at Inner Vision Day Support. Center notes confirm he settled into the morning routine well, contributing to group discussion and completing structured tasks with peers. He packed his lunch on his own before leaving, a routine he now handles with minimal prompting (Outcome 4). He returned to the residence by taxi at 4:00 PM with no health concerns reported.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Calm, Polite

Routine Support Protocols & Skill Practice: Steven returned to the home at 4:00 PM and eased into a relaxed evening after a short check-in with staff (Routine Support 2). He spent time on his mobile games and strummed a few songs on his guitar. When it was time to cook, he assembled a garden salad with grilled chicken, measuring dressing to stay on his diet (Outcome 4). Evening medications were provided as ordered, with water, and logged (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). He remained safe and settled through the evening.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Overnight Clinical Observation: Steven wrapped up his personal-care routine without assistance and was in bed by his usual hour. Bedtime medications were taken with water per order (Routine Support 3). Staff performed regular overnight rounds, watching for seizure activity, respiratory changes, and stability (Risk 2). He slept without interruption and woke calm and rested. No seizures, falls, or skin-integrity concerns were observed, and vitals stayed stable.

May 29, 2026 (Friday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Day Support Staff | Observed Mood: Relaxed, Talkative

Structured Program & Peer Interaction: A taxi arrived promptly at the group home at 7:45 AM to take Steven to Inner Vision Day Support. Center documentation reflects that he integrated smoothly into the structured schedule, joining socialization blocks, community-inclusion tasks, and guided behavioral activities. His self-packed lunch met his dietary plan, reflecting continued independent-living progress (Outcome 4). He came back to the residence safely by taxi at 4:00 PM.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Pleasant, Settled

In-House Recreation & Meal Preparation: Steven was welcomed home at 4:00 PM and transitioned smoothly into the evening. Staff offered a supportive check-in about his day (Routine Support 2). He watched an episode of a favorite series while unwinding on the couch before dinner, then assisted with a lean turkey stir-fry, carefully watching serving sizes per his diabetic plan, paired with a side salad (Outcome 4). Evening medications were administered on schedule with water (Routine Support 3). Staff kept water within reach and reminded him to sip throughout the evening (Risk 1). Staff watched for seizure activity and safe swallowing while he ate (Risk 2, Potential Risk 2). The evening passed calmly with no issues.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Calm, Sleepy

Nighttime Monitoring & Medication Compliance: Following an independent nighttime hygiene routine, Steven retired to bed at his scheduled hour. The nightly medication pass was verified and given with water (Routine Support 3). Overnight surveillance covered seizure status, respiration, and blood-sugar stability for the full shift (Risk 2). He remained asleep and settled across the entire block. Steven showed no seizures, no falls, and no diabetic or respiratory concerns.

May 30, 2026 (Saturday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Happy, Cooperative

Structured Weekend Outing: Rahel supported Steven with a weekend outing to the community center's open gym for light, low-impact movement and social time (Outcome 2). Steven practiced polite social exchanges and stayed patient and cooperative during the outing. Staff monitored for seizure activity and steered him toward water over sugary drinks throughout (Risk 1, Risk 2). No medical or behavioral concerns were observed.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Settled, Friendly

Evening Wellness & Cooking Practice: Home at 4:00 PM, Steven eased into his evening after a brief 1-on-1 check-in with staff (Routine Support 2). He flipped through a car magazine and shared a few pictures with staff. At dinnertime, Steven and staff cooked together at the home, preparing baked chicken thighs with roasted zucchini and a side salad, checking portions with staff, applying the portion habits he has been practicing (Outcome 4). The evening medication pass was completed accurately with water (Routine Support 3). Water intake was met through verbal reminders and a preferred sugar-free flavor packet (Risk 1), and staff observed him for any seizure or swallowing difficulty at the table (Risk 2, Potential Risk 2). No concerns were noted across the block.

Shift (Start and End Time): 12am-8am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Peaceful, Sleepy

Nighttime Monitoring & Medication Compliance: Steven independently completed his bedtime hygiene care and lay down on schedule. Bedtime medications were taken with water per order (Routine Support 3). Hourly welfare checks tracked breathing, seizure activity, and diabetic stability across the shift (Risk 2, Potential Risk 3). He slept soundly through the full night with no disruptions. Steven showed no seizures, no falls, and no diabetic or respiratory concerns; he stayed secure under continuous supervision.

May 31, 2026 (Sunday)

Shift (Start and End Time): 8am-4pm

Assigned Provider: Hana Tafesse, DSP | Observed Mood: Happy, Cooperative

Weekend Community Integration: Steven's weekend leisure block was led by Hana, who took him to a neighborhood park for a scenic walk and fresh air (Outcome 2). Steven practiced polite social exchanges and stayed patient and cooperative during the outing. Seatbelt safety was observed during transport and staff prompted regular water intake (Risk 1). No medical or behavioral concerns were observed.

Shift (Start and End Time): 4pm-12am

Assigned Provider: Nathaniel Yohannes, DSP | Observed Mood: Relaxed, Cooperative

Evening Skill Practice & Health Monitoring: Steven arrived home at 4:00 PM and settled in comfortably. Staff completed a communication check-in, giving him a safe space to talk through his day (Routine Support 2). He flipped through a car magazine and shared a few pictures with staff before joining staff in the kitchen, where Steven and staff cooked together at the home. He grilled a lean burger patty with a side salad, following his diabetic portion rules (Outcome 4). Evening medications were provided as ordered, with water, and logged (Routine Support 3). Staff kept water within reach and reminded him to sip throughout the evening (Risk 1), and staff monitored closely for any seizure or choking signs during the meal (Risk 2, Potential Risk 2). He remained calm and safe throughout.

Shift (Start and End Time): 12am-8am

Assigned Provider: Rahel Tafesse, DSP | Observed Mood: Peaceful, Sleepy

Overnight Clinical Observation: Steven completed his nighttime hygiene routine independently and retired to bed on schedule. Bedtime medications were taken with water per order (Routine Support 3). Continuous monitoring was maintained through the night for seizures, breathing, and diabetic baselines (Risk 2, Potential Risk 3). He had a restful, uninterrupted night of sleep. The shift passed without any seizure activity, falls, or medical concerns. He remained safe and secure under residential care.

REGULATORY STAFF SIGNATURE & ATTESTATION KEY

By signing below, the Direct Support Professionals (DSPs) and Residential Program Manager legally attest that the clinical data, behavioral observations, medication administrations, and individualized service plan (ISP) support notes recorded in this multi-page document are accurate, true, and compiled in absolute compliance with state healthcare review boards and waiver guidelines.

Staff Printed Name	Title	Staff Initials	Legal Signature	Date Signed
Rahel Tafesse	Direct Support Professional (DSP)	RT		
Hana Tafesse	Direct Support Professional (DSP)	HT		
Nathaniel Yohannes	Direct Support Professional (DSP)	NY		
Hiwot Tedla	Residential Program Manager (PM)	HTM		